



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : رشا جمال الدين مصطفى -

العنوان : الجيزة

الحي / المدينة : الجيزة

المحافظة : الجيزة

تاريخ تسجيل الإستمارة : 30-09-2026

رقم البطاقة : 28011102103481 رقم التليفون : 01115685940

العمر (تاريخ الميلاد) : 10-11-1980 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /..... /د. /..... /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: رشا جمال الدين مصطفى - National ID: 28011102103481	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Bilateral mammogram and breast ultrasound
--

التاريخ :	الوقت :
05-07-2026	-- --:--
	الطبيب المعالج : د/شيماء لاشين



MDT FORM

11 / 3 / 2026 G

Name	سماح محمد علي	MRN	DMS 8301
Age	45 yr.	ID	CA 111. C1. 24.1
Menopausal status	Pre-menopausal	Phone	01115685940
NRS SCORE		Marital status	Married
BMI		Family history	-ve
CO morbidity	HTN	REASON OF MDT	New Case

Clinical Data

BX: IDC GIII

ER: 018 PR: 018 HER 2: 0 Ki67: 90%.

Sonomammography:

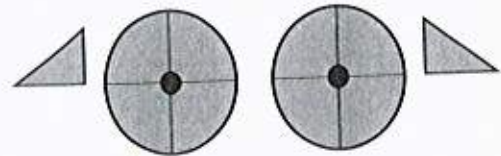
- Rr 2 o'clock mass 2.5x2.6cm
① satellite 8x7mm ② Multiple

Mets Workup:

- CT CAP → Bilateral acetabular lytic areas.
- Bone scan → free

R

L



pathological
Axix

TNM

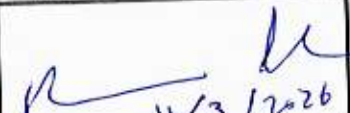
Recommendations:

MDT OUTCOME & PLAN Fr:

- ① Echo cardiography
- ② Clip insert in Rr 2 o'clock mass
- ③ Portacath insert
- ④ Re start Keynote S22 protocol (Taxol/Cabox12)

SIGNATURE

consider Rr WLE & MWP.

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
 11/3/2026					

Name:	رشا جمال الدين مصطفى القاياتي	
MR. :	8301	
Detailed Medical Report		
		السيد
تحية طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
نتشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه The patient's weight was recorded as 98 kg and height 157 cm in preparation for week 1 of TAXOL and CARBOPOPLATIN AUC 2, with KEYTRUDA to be added after laboratory results. The patient was diagnosed with left-sided breast cancer, clinical stage T2 N1 M0, IDC grade 3, triple-negative, Ki-67 90 %. Surgical clips were placed, and acetabular lesions were noted. For week 2, the plan was TAXOL 160 mg and CARBOPOPLATIN AUC 2. On examination, a left upper outer quadrant mass measured 4 × 5 cm with a left axillary lymph node 1 × 1 cm. Weight 98 kg, height 157 cm, body surface area 2 m², and adjusted body weight 69 kg were documented for week 2; TAXOL 160 mg and CARBOPOPLATIN AUC 2 were administered after laboratory values were confirmed on 16 April. Pharmacy visit confirmed receipt of week 3 medications: TAXOL 160 mg plus CARBOPOPLATIN AUC 2 (dose 250 mg) with labs performed on 16 April and accepted. The patient experienced grade 2 peripheral neuropathy; for week 4, TAXOL 145 mg and CARBOPOPLATIN AUC 2 were prescribed, with KEYTRUDA added as neoadjuvant therapy pending thyroid function testing. Pharmacy visit for week 6 documented administration of TAXOL 145 mg, CARBOPOPLATIN AUC 2, and G-CSF, with labs from 30 April reviewed and accepted. For week 7, the regimen included TAXOL 145 mg, CARBOPOPLATIN AUC 2, G-CSF, and KEYTRUDA, pending laboratory results for weeks 8 and 9, with pharmacy coordination noted. Clinical pharmacy note recorded administration of week 9 TAXOL 145 mg and CARBOPOPLATIN AUC 2. During week 11, the patient received TAXOL after laboratory review and was evaluated for continuation of cycle 3 versus 4 of KEYTRUDA. Pharmacy note for week 12 indicated administration of TAXOL 140 mg and CARBOPOPLATIN AUC 2. The patient was evaluated for a suspected renal stone and referred to urology.		
تحريراً في ,		
	الوقت :	التاريخ : 20-09-2026
dina makram		الطبيب المعالج :
يعتمد ,		

A CT urogram was performed for urology consultation due to oliguria.

For cycle 4 of KEYTRUDA and week 12, TAXOL 145 mg and CARBOPOPLATIN AUC 2 were administered; examination revealed no palpable breast masses, and thyroid profile testing was planned pending CT urogram results.

The patient's comorbidities include diabetes mellitus and hypertension; ejection fraction was 67 % on 13 July. CT urogram on 13 July showed bilateral renal calculi. The patient received neoadjuvant ENDOTAXAN 120 mg/m² (total 1200 mg) and plans for cycle 5 of KEYTRUDA when available; examination again demonstrated no breast masses.

The current medication list includes CYMBATEX 30 mg once daily for 30 days, THIOTACID 600 mg once daily for 30 days, and METHYLTECHNOSTRIPS applied once daily for 30 days.

Name : RASHA GAMAL EL DEEN

Date : 10-02-2026 11:53

MOUSTAFA

Branch : Portable

Medical No. : GRI-28011102103481

Ref.Physician Prof : CT CAP+C

Ins.Company :

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN
AND PELVIS REVEALED:**

Known case of CA breast.

I- CHEST:

- Left breast lesion is seen measuring 45 x 28 mm for sono-mammography.
- Multiple left axillary pathologically enlarged lymph nodes largest measuring 23 x 23 mm.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Mildly enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Few bilateral acetabular lytic areas are noted.

Opinion:

- ❖ Left breast lesion with left axillary lymphadenopathy for sono-mammography.
- ❖ Mild hepatomegaly.
- ❖ Few bilateral acetabular lytic areas for bone scan.

Dr. Sherif Wagdy

Dr. Sherif Wagdy



Presidential Initiative for Women's Health
Pathology Lab - Cairo University Hospitals

المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي
مستشفيات جامعة القاهرة



PATHOLOGY REPORT

Name: رشا جمال الدين مصطفى
Patient's I.D.: 28011102103481
Age: 46 Y.

Pathology Number: 11/2026
Specimen Received Data: 12 / 01 /2026
Date Report Issued: 22/ 01 /2026

Clinical data: Left breast UOQ mass, BIRADs 5.
Nature of the specimen: U.S. Guided Tru Cut Biopsy.

GROSS EXAMINATION: Tissue core biopsy, totally submitted.

MICROSCOPIC EXAMINATION: Serial sections examined revealed cores of breast tissue showing an invasive malignant tumor formed of solid groups and few poorly formed tubules of markedly anaplastic epithelial cells. Focal high grade DCIS was seen. Surrounding desmoplastic stroma with 5% TILs with areas of necrosis was seen.

IMMUNOHISTOCHEMISTRY PROCEDURE: Sections from paraffin block were treated with antigen retrieval solution and then treated with monoclonal antibodies for estrogen, progesterone, Her-2/neu AND Ki-67 (CONFIRM™ anti-Estrogen Receptor (ER) (SP1) Rabbit & CONFIRM™ anti-Progesterone Receptor (PR) (1E2), CONFIRM™ anti-HER-2/neu (4B5) Primary Antibody), and CONFIRM anti-Ki67 (30-9) Primary Antibody), using Ventana iView™ system. DAB was used as chromogen and hematoxylin as counter stain.

IMMUNOHISTOCHEMISTRY RESULTS:

Examination of section treated for estrogen receptors revealed negative nuclear staining of tumor cells by chromogen.
Examination of section treated for progesterone receptors revealed negative nuclear staining of tumor cells by chromogen.
Examination of section treated for Her-2/neu revealed negative membrane staining in tumor cells by chromogen (score 0).
Examination of section treated for Ki67 revealed positive nuclear stain in average of $\pm 90\%$ of tumor cells.

DIAGNOSIS: Left Breast Mass, U.S. Guided Tru Cut Biopsy:

- Invasive Duct Carcinoma, NST, Grade III.
- **Negative** For Estrogen Receptors, Score 0/8.
- **Negative** For Progesterone Receptors, Score 0/8.
- **Negative** For Her-2/neu Over-Expression (Score 0).
- **90%** Ki-67 Proliferative index.

PROF. DR. PASSANT ESSAM SHIBEL
Dr. MIRAN ATIF

Handwritten signature of Prof. Dr. Passant Essam Shibel



Patient Name	soheer Saad Ibrahim Ali	PID	1021012330 2314	D/T	22-01-2026
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SONOMAMMOGRAPHY OF BOTH BREASTS

Procedure: Digital mammography and real time ultrasonography of both breasts were performed.

Findings:

- Both breasts display normal skin thickness with normal appearance of the nipples.
- The breast parenchyma shows a heterogeneously dense fibroglandular pattern (ACR breast density 3).
- Normal appearance of the subcutaneous and retromammary fat on both sides.
- The right breast shows a well defined, oval hypoechoic lesion, 10 O'clock position, cm from the nipple, measuring 5x2.2 mm.
- The left breast shows another lesion with same characteristics being well defined, oval hypoechoic lesion, 12 O'clock position, 3 cm from the nipple, measuring 4x1.5 mm.
- The left breast also shows clustered microcysts at 1 o'clock 5 cm from the nipple, measuring collectively 10x3.8 mm.
- No suspicious solid or abnormal calcifications were detected in the both breasts.
- Bilateral reactionary looking axillary lymph nodes, being oval in shape with preserved central fatty hilum, yet increased cortical thickness reaching 3.1 mm on the right side and 4.5 mm on the left side.

Impression:

BILATERAL BENIGN LOOKING BREST LESIONS LIKELY FIBROADENOMAS (BIRADS 3) FOR FOLLOW UP.
BILATERAL REACTIONARY LOOKING AXILLARY LNS YET WITH INCREASED CORTICAL THICKNESS FOR FOLLOW-UP.

Much obliged
R/Youmna Elsheikh
Abanoub Samuel, MSc

Patient's Name : rasha gamal
Date : 6-1-2025

Bilateral BREAST SONOMAMOGRAPHY

- The skin, subcutaneous tissue, nipple and areolae of both breasts are seen within normal. the breast parenchyma is mainly fatty with scattered glandular tissue
- ACR B Mammogram shows an irregular shaped hyperdense mass lesion with spiculated seen at the upper outer quadrant of left breast at about 2 o'clock with suspicious axillary lymph node
- Complementary Ultrasound shows an irregular hypoechoic solid mass with indistinct margins, echogenic halo sign & acoustic shadowing seen at the upper outer quadrant opposite to 2 O'clock, measuring 2.6 x 2.5 x 2 cm and about 1.2 cm from the skin and 5 cm from nipple areolar region surrounded with mild parenchymal distortion with small adherent satellite lesion measures 8 x 7 mm with no vascularity by color doppler with multiple axillary enlarged lymph nodes one of them seen measures 2.33 x 1.5 cm with eccentric hilum and focal cortical bulging measures 1 cm sonographic features of suspicious mass lesion with suspicious axillary lymph node
BIRADS V For FNA
- No right sided breast solid, cystic mass lesion
- Right sided non specific reactive lymphadenitis

OPINION:

⇒ Left sided A highly suspicious lesion with axillary suspicious lymph node (BIRADS V).... For FNA.

kindest Regards
Dr/ christen helmy



S C A N

Findings:

- No abnormal tracer uptake is seen over the surveyed skeleton that would account for bone metastases.
- Warm tracer uptake noted in left side of the lower jaw, likely dental or clinical correlation.
- Normal visualization of both kidneys.

Impression:

- No scintigraphic evidence of osseous deposits.

Dr. Eman Ashraf, MD

Dr. Omnia Alaa, MSc

PATIENT NAME رشا جمال الدين مصطفى			Registered 05-08-2026 14:31:28	Collected 05-08-2026 14:34:45
Visit Number 71526511703	Age 46 Year	Gender Female	Authenticated 26-08-2026 10:12:56	Printed 12-09-2026 13:48:38
			Referred By Prof : -	Client ID 55452

MOLECULAR BIOLOGY REPORT

BRCA 1.2 WHOLE GENE SEQUANCING / BLOOD (Astrazneca)

Result : No Pathogenic variants detected in BRCA1 or BRCA2 genes sequencing.

Background

- Hereditary Breast Cancer accounts for about 5-10% of female breast cancer and 4-40% of male breast cancer. The most common cause is mutations in the BRCA1 & BRCA2 genes which are tumor suppressor genes located on chromosomes 17 and 13 respectively.
- Pathogenic variants of these genes are inherited in an autosomal dominant manner and are associated with hereditary breast and ovarian cancer (HBOC) syndrome.
- HBOC syndrome increases the risk for breast (female and male), ovarian, fallopian tube, peritoneal, pancreatic, and prostate cancers. Additionally, BRCA2 carriers may be at increased risk for melanoma.

Method

- Genomic DNA was extracted from the patient's specimen. Libraries were prepared according to manufacturer's protocol then next-generation sequencing was performed on the Illumina MiSeq platform using CE-IVD BRaCA assay
- The assay analyzes sequencing of all exons and immediately adjacent intronic regions of BRCA1 & BRCA2 genes.
- Raw sequence data analysis, including base calling, demultiplexing, alignment to the hg19 human reference genome (Genome Reference Consortium GRCh37), and variant calling is done using standalone software. Variants are interpreted using publicly available databases such as Exac, GnomAD, dbSNP, ClinVar, Brca exchange. Variants are classified according to the American College of Medical Genetics and Genomics (ACMG) guidelines.
- Benign and likely benign variants are not included in this report.
- Variants of unknown significance classification may change over time.

Limitations

- Large rearrangements may not be detected, variants in regions with high GC content or pseudogenes may give false negative or false positive results.

Rania Darwish

Dr. Rania Darwish
Prof of Clinical & Chemical Pathology,
Faculty of Medicine, Cairo University 1-2

اسم الفرع نرسا
مدير الفرع د. امنيه خاله سيد عريض
مدير الوحدة ا.د رانيا درويش
طبيب الوحدة ا.د. عطياء النش

رئيس مجلس الادارة
ا.د. مؤمنه كامل
مدير المعامل
ا.د. هند الشربيني

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Referred By Prof : -			Client ID 55452	

- The test result is for clinical reference and clinicians should interpret it in the context of the patient's condition, clinical findings, family history, other lab investigations, and international guidelines for treatment.

References

- Yoshida, Reiko. "Hereditary breast and ovarian cancer (HBOC): Review of its molecular characteristics, screening, treatment, and prognosis." Breast Cancer 28.6 (2021): 1167-1180.
- Richards, Sue, et al. "Standards and guidelines for the interpretation of sequence variants: a joint consensus recommendation of the American College of Medical Genetics and Genomics and the Association for Molecular Pathology." Genetics in Medicine 17.5 (2015): 405-423.

Rania Darwish

Dr. Rania Darwish
Prof of Clinical & Chemical Pathology,
Faculty of Medicine, Cairo University 2-2

12.1

Patient ID: 93901	Patient Name: English: Rasha Gamal Aldin Mostafa
رقم الملف 93901	2026/03/18 10:14:16 AM :Report Date

Ultrasound Guided clip insertion:

Clinical summary: Pathologically proven left breast invasive duct carcinoma, for clip insertion.

Technique:

- Under complete sterile conditions, 3ml of local anaesthetic were administered at the site of concern of the Left breast.
- One metallic clip is inserted within the left UOQ malignant mass. The reported satellite is inseparable from the mass.
- This was confirmed by mammography CC & ML views.

Best Regards
Dr. Sherihan Fakhry, MD

Name : RASHIA GAMAL EL DEEN
MOUSTAFA

Date : 10-02-2026 11:53

Branch : Portable

Medical No. : GRI-28011102103481

Ref.Physician Prof : CT CAP+C

Ins. Company :

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- No ascites.
- Few bilateral acetabular lytic areas are noted.

Opinion:

- ❖ Left breast lesion with left axillary lymphadenopathy for sono-mammography.
- ❖ Mild hepatomegaly.
- ❖ Few bilateral acetabular lytic areas for bone scan.

Dr. Sherif Wagdy

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المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي
مستشفيات جامعة القاهرة



PATHOLOGY REPORT

Memo: رجا جمال الدين مصطفى

Patient's I.D.: 28011102103481

Age: 46 Y

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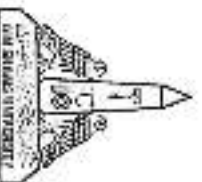
Examination of section treated for Ki67 revealed positive nuclear stain in average of +90% of tumor cells.

DIAGNOSIS: Left Breast Mass, U.S. Guided Tru Cut Biopsy.

- Invasive Duct Carcinoma, NST, Grade III.
- Negative For Estrogen Receptors, Score 0/8.
- Negative For Progesterone Receptors, Score 0/8.
- Negative For Her-2/neu Over-Expression (Score 0).
- 90% Ki-67 Proliferative index.

PROF. DR. PASSANT ESSAM SHIBEL
Dr. MIRAN ATIF

Signature
Date



Patient Name	soheer Saad Ibrahim Ali	PID	1021012330 2314	D/T	22-01-2026
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BILATERAL REACTIONARY LOOKING AXILLARY LNS YET WITH INCREASED CORTICAL THICKNESS FOR FOLLOW-UP.**

Much obliged
R/Yomna Elsheikh
Abanoub Samuel, MSc

Patient's Name : rasha gamal
Date : 6-1-2025

Bilateral BREAST SONOGRAPHY

- The skin, subcutaneous tissue, nipple and areolae of both breasts are seen within normal, the breast parenchyma is mainly fatty with scattered glandular tissue
- ACR B
- Mammogram shows an irregular shaped hyperdense mass lesion with speculated seen at the upper outer quadrant of left breast at about 2 o'clock with suspicious axillary lymph node

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Kindest Regards
Dr/ christen helmy



S C A N

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- Warm tracer uptake noted in left side of the lower jaw, likely dental or clinical correlation.
- Normal visualization of both kidneys.

Impression:

- No scintigraphic evidence of osseous deposits.

Dr. Eman Ashraf, MD

Dr. Omnia Alaa, MSc